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HEMATOLOGY HIGHLIGHTS From ASH 2025

Insight Corner



Month: January 2026

Presenter: Himanshu Sehgal



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- Highlights from AACR-25 (**May, 25**)
- US Tariff impact on Pharma industries (**Jun, 25**)
- EULAR-25: Key insights of Psoriatic Arthritis (**Jul, 25**)
- Antibody Drug Conjugate: The next big investment in Oncology (**AUG,25)**
- Animal Health (**Oct ,25)**
- Obesity deep dive (**Nov, 25**)
- GLP-1 and GIP (**Dec, 25**)

Additional: 1 Beautiful Bill



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67th AMERICAN SOCIETY OF HEMATOLOGY Annual Meeting

Multiple Myeloma

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Why ASH 2025 matters in Multiple Myeloma??

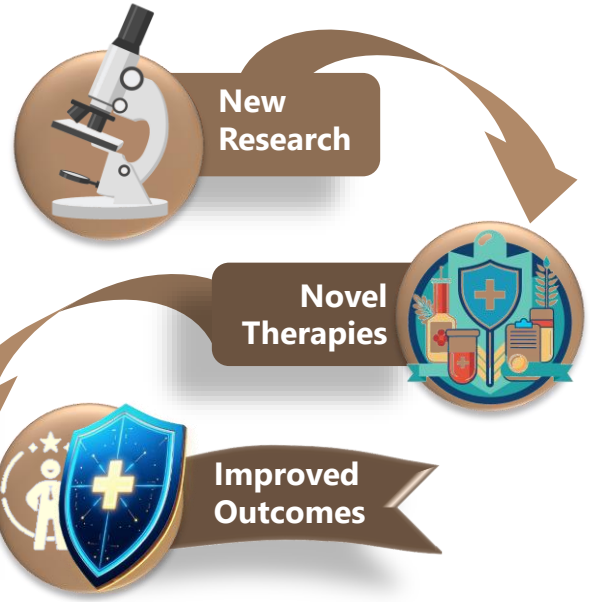
At this year's ASH Annual Meeting

Nearly 2,000 of 9,000 abstracts

Focused on Multiple Myeloma







Underscoring Rapid Progress Across the Disease Continuum

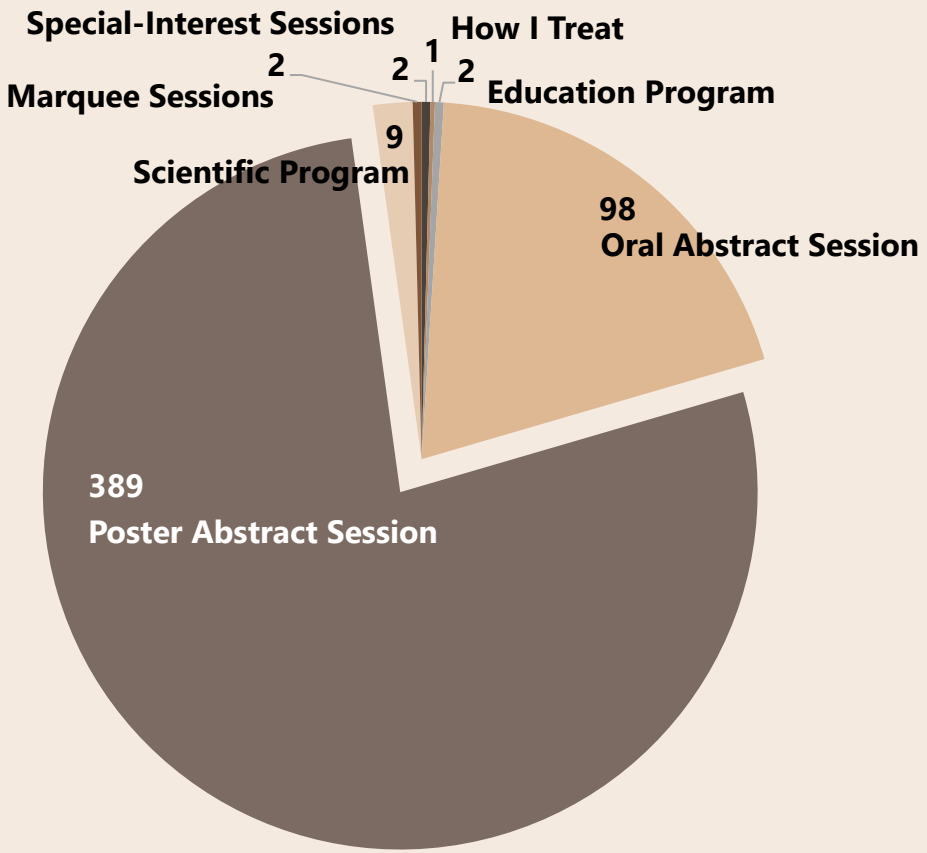
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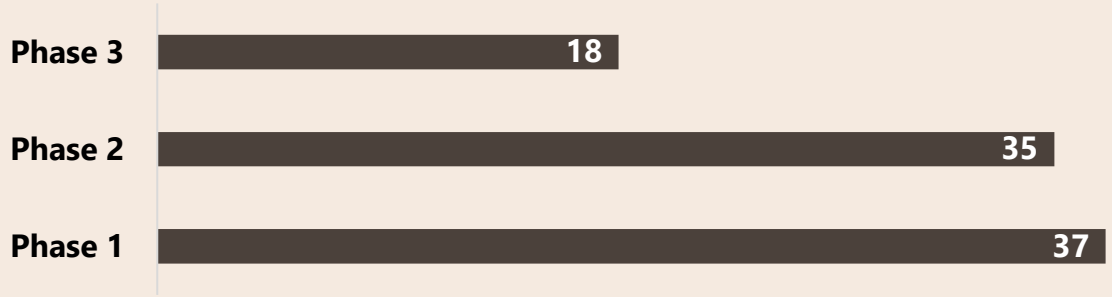
Multiple Myeloma

ASH 2025

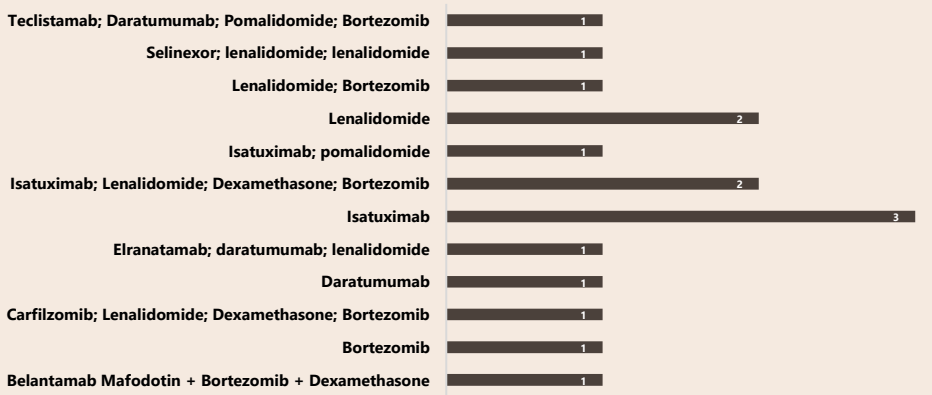
Number of Sessions



Distribution by Clinical Phases



Molecules in Phase 3



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How ASH'25 is reshaping Multiple Myeloma care??

1. Practice-changing trial data in multiple myeloma, especially MAJESTEC-3 and other key studies, showed major survival and disease-control benefits.
2. Powerful immunotherapies (bispecific antibodies, CAR T, tri-specifics) are moving earlier in treatment, not just for end-stage disease.
3. Minimal residual disease (MRD) negativity is being cemented as a main treatment goal and guide for response-adapted therapy.
4. New combinations (for example, teclistamab plus daratumumab) delivered unprecedented depth of response in early relapse, likely changing second-line standards.
5. Emerging *in vivo* CAR-T and *off-the-shelf* antibodies promise wider access, shorter treatment duration, and potential treatment-free periods for some patients.

This presentation is non-exhaustive. For additional details or clarifications, please feel free to contact us at bd@octavusconsulting.com

Novel Targeted Drugs and Gene Therapies

Inobrodib Dose Optimization

Inobrodib (CCS1477) combined with pomalidomide, and dexamethasone shows dose-Optimisation efficacy in Phase II study for r/r MM

Gene Therapy Breakthrough

KLN-1010 gene therapy in inMMyCAR trial induces rapid MRD-negative responses, representing cutting-edge cellular engineering

Targeted Therapy

High efficacy of Sonrotoclax (BGB-11417) with carfilzomib and dexamethasone in t(11;14)-positive r/r MM subgroup



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Key Highlights Presented at ASH 2025

- ✓ *Increasing treatment options for MM is a major focus, with new combinations and frontline therapies (Eg. Tec-dara for r/r MM , KYPROLIS + lenalidomide + dexamethasone, Selinexor + lenalidomide for NDMM)*
- ✓ *Bi-specific antibodies and CAR T-cell therapies, potentially transforming treatment paradigms*
- ✓ *This year's data highlights pairing anti-CD38 therapies with bi-specifics, including the late-breaker LBA-6 for Tec-Dara, featuring results from the MajesTEC-3 phase 3 study emphasising combination works better than current standard treatments.*
- ✓ *Gene Therapy Breakthrough - Early results from the Phase 1 inMMyCAR™ study demonstrate ongoing MRD-negative responses in all patients with a tolerable safety profile*

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Bi-specific Breakthroughs

Game-Changing Clinical Data

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Teclistamab + Daratumumab

Johnson & Johnson

Key Outcomes

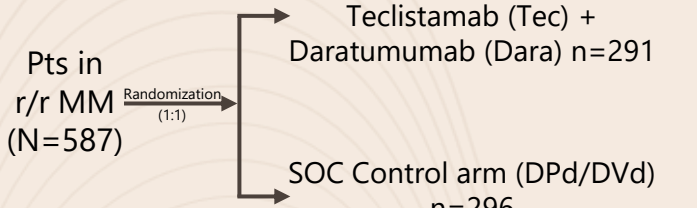
- ✓ Phase 3 **MajesTEC-3** trial (NCT05083169) (587 r/r MM pts., 1–3 prior lines) shows Tec-Dara greatly improves PFS and OS vs standard DPd/DVd, with mPFS not reached vs 18.1 mos and higher ≥CR and MRD-negative rates
- ✓ Safety profiles are broadly comparable between arms (similar grade 3/4 and grade 5 TEAEs), though infections are more frequent with Tec + Dara but considered manageable, leading to low treatment discontinuation due to adverse events
- ✓ Overall, Tec + Dara emerges as a highly effective, off-the-shelf immunotherapy that can serve as a new standard of care for r/r MM as early as first relapse

About Teclistamab

Teclistamab is a ready-to-use BCMA×CD3 bispecific antibody for multiple myeloma that redirects T cells to kill myeloma cells.

- It delivers deep and durable responses with a manageable safety profile and offering an effective, accessible alternative to CAR-T therapy, including in earlier relapses
- Teclistamab is already received accelerated approval from USFDA for r/r MM and marketed by Janssen (J&J) under the brand name **TECVAYLI**

Study Design



Inclusion Criteria

- 1–3 prior lines of therapy (LOT)
- If 1 prior LOT → lenalidomide-refractory

Exclusion Criteria

- Prior BCMA-directed therapy
- Refractory to anti-CD38 therapy
- Prior anti-CD38 exposure allowed

Endpoints & Assessments

- 1EP: PFS by IRC
- 2EP: ORR, >CR, OS, Safety & tolerability

Efficacy

Endpoints	**Tec + Dara** (n=291)	**SOC (DPd/ DVd)** (n=296)
mPFS (mos)	NR	18.1
36 mos. PFS rate (%)	83.4	29.7
ORR (%)	89.0	75.3
≥CR rates (%)	81.8	32.1
36 mos. OS rate (%)	83.3	65.0
MRD-negativity (%)	58.4	17.1

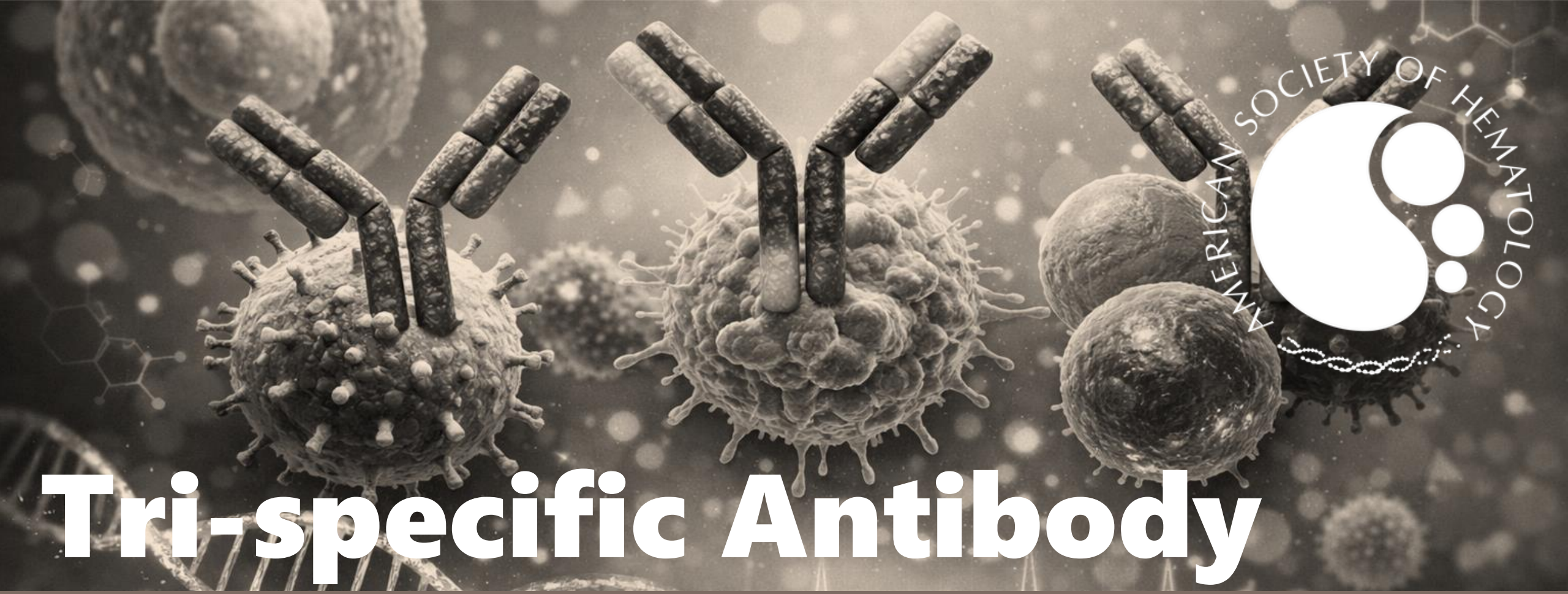
Safety

Endpoints	**Tec + Dara** (n=283)	**SOC (DPd/ DVd)** (n=290)
Pts. Remained on study (%)	71	28.3
mTD (mos)	32.4	16.1
Incidence of grade 3/4 (%)	95.1	96.6
Grade 5 TEAEs (%)	7.8	6.2
Treatment discontinued due to TEAEs (%)	4.6	5.5
Any grade infection (%)	96.5	84.1

Abb.: r/r MM: relapsed/ refractory multiple myeloma; mPFS: median progression-free survival; OS: overall survival; CR: complete response; MRD: minimal residual disease; TEAEs: treatment-emergent adverse events; IRC: Independent Review Committee; ORR: overall response rate; mTD: median treatment duration I **Sources:** ASH 2025 LBA-6

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Tri-specific Antibody

Next-Generation

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Ramantamig

Johnson & Johnson

About Ramantamig

Ramantamig (JNJ-5322) is a next-generation tri-specific T-cell–engaging antibody that binds BCMA X GPRC5D (on myeloma cells) X CD3 (on T cells), designed to overcome tumor heterogeneity in r/r MM.

Study Design

Pts in r/r MM
Phase 1

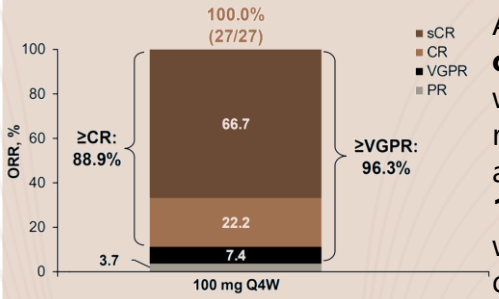
Part 1
- Dose Escalation
- Identify RP2D & Schedules

Part 2
- Dose Expansion
- Patients treated at RP2D

- RP2D was identified as 100mg SC Q4W, following a single 5 mg subcutaneous step-up dose
- Present updated safety results (N=36; mFU= 17.0 mos) and efficacy results in triple-class exposed patients naive to T-cell redirecting therapy (N=27; mFU=17.8 mos) at the RP2D from the ongoing phase 1 study. DCO: Sep'25

Efficacy

At the **RP2D**: **ORR** of **86.1%** (**n=31/36**), **≥CR** rate of **75.0%**, and **MRD-negativity** rate of **100.0%** at 10 –5 (n=10/10) and 10 –6 (n=7/7)



At the **RP2D**, **triple-class**–exposed pts who were **naive** to **T-cell** -redirecting therapies achieved an **ORR** of **100.0%** (**n=27/27**) , with responses further deepening since the prior analysis.

Key Outcomes

- ✓ At the fixed RP2D of 100 mg Q4W, ramantamig (JNJ-5322) shows a favorable safety profile Vs. BCMA/GPRC5D bispecific antibodies, with low oral adverse events, minimal weight loss, low CRS rates, no ICANS, and manageable infections using optimized infection-management strategies
- ✓ The redesigned GPRC5D and CD3 binding domains, together with a single SUD and fixed Q4W dosing, likely contribute to this favorable safety profile, allowing convenient single step-up outpatient dosing and no grade ≥2 CRS or grade 3 oral events
- ✓ At the RP2D, ramantamig achieves a 100% overall response rate with deepening responses over time, including ≥CR in 88.9% of triple-class–exposed, T-cell–redirecting-therapy–naïve patients, and maintains robust, durable responses with high MRD negativity

Safety

At the RP2D remained consistent with prior findings, CRS 53% of pts → Grade 1: 42% (Incl. those who received prophylactic tocilizumab); Grade 2: 11%

Safety (N=36)	% of total	TEAEs	Any grade	Grade ¾
Hematologic	≥10	Lymphopenia	47.2	44.4
		Neutropenia	44.4	33.3
Non-Hematologic	≥30	Infections	77.8	33.3
		Skin (non-rash)	66.7	0.0

Abb.: r/r MM: relapsed/ refractory multiple myeloma; RP2D: recommended Phase 2 Dose; SC: subcutaneous; Q4W: every 4 week; mFU: median follow-up; DCO: date cut-off; ORR: overall response rate; CR: complete response; MRD: minimal residual disease; CRS: cytokine release syndrome; ICANS: immune effector cell–associated neurotoxicity syndrome; SUD: step-up dose I **Sources:** ASH 2025 PN-4042

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CAR-T Therapy

The Cancer Game Changer

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Anitocabtagene Autoleucel

Kite
A GILEAD Company

About Anitocabtagene Autoleucel

Anitocabtagene autoleucel (anito-cel) is an autologous, genetically engineered T-cell therapy belonging to the class of chimeric antigen receptor (CAR) T-cell therapies. This CAR enables T cells to recognize a specific target antigen on diseased cells with high precision.

About iMMagine-1

- Phase 2 (NCT05396885)
- at least 3 prior lines of therapy (4L+) r/r MM

N=117	% of Pts
Triple refractory	87
Penta refractory	41
Extramedullary disease	18
High risk cytogenetics	40

- mFU: 15.9 mos
- DCO: Oct'25

Efficacy

Endpoints (%)	PFS	OS
@12mos	82.1	94
@18mos	67.4	88
@24mos	61.7	83

- ORR by IRC: 96%
  - sCR or CR: 74%
- MRD-negativity @mT1mos: 95%
- mPFS and mOS not reached

Safety

- CRS: 86% of pts
  - even 83% of pts: no CRS or Grade 1 CRS (fever only)

Most common Hematologic AEs	
Neutropenia	71%
Anemia	28%
Thrombocytopenia	26%

- Grade 3 or higher inf.: 9%

Key Outcomes

- ✓ Efficacy is robust, with PFS rates of 82.1% and OS of 94% at 12 mos
- ✓ Safety shows mainly low-grade CRS and manageable hematologic AEs, positioning it as an advanced precision immunotherapy with potent, selective activity
- ✓ **These results supports anito-cel for a potential U.S. launch in 2026**

Abb.: r/r MM: relapsed/ refractory multiple myeloma; mFU: median follow-up; DCO: date cut-off; mPFS: median progression-free survival; mOS: median overall survival; MRD: minimal residual disease; CR: complete response; mT1mos: median time 1 month; CRS: cytokine release syndrome; AEs: adverse effect | **Sources:** ASH 2025 PN-256; Gilead PR Dec'25 | **Data captured based on press release, which has a later data cutoff (Oct'25) than ASH abstract (May'25), so it reflects the most recent results.**

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Opportunity Assessment

How big is the Market



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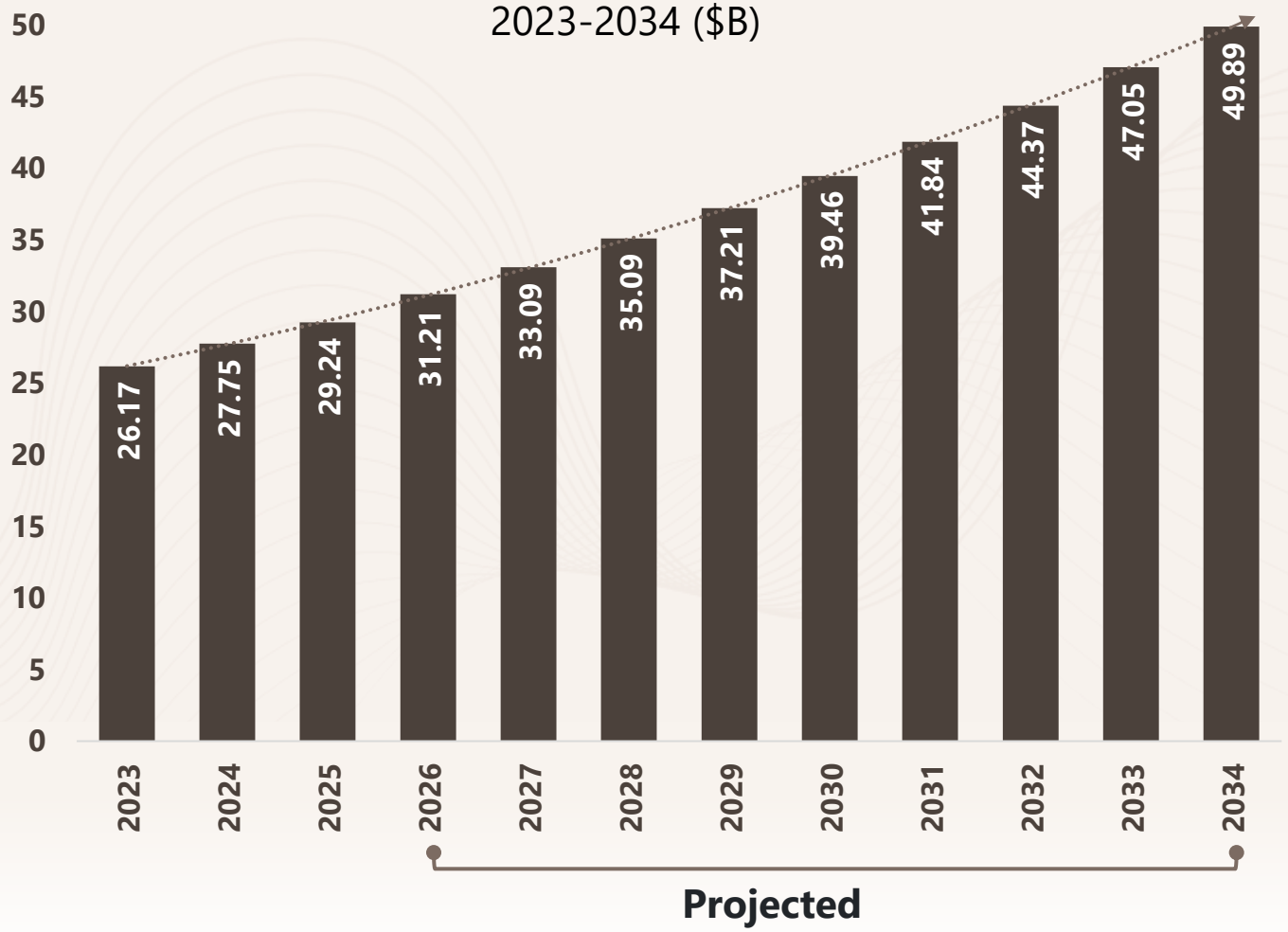
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Market Outlook

- The global multiple myeloma market size was estimated at $29.24 in 2025 and is projected to increase to about $31.21B in 2026 to $44.19B by 2034, exhibiting a CAGR of ~6% during the forecast period
- Long-range forecasts indicate the market could reach approximately $49.89B by 2034, implying sustained demand for diagnostics, therapies, and supportive care
- This growth also mirrors rising diagnosis rates, longer patient survival, and expanding access to advanced treatments in emerging regions



Global Multiple Myeloma Market Size Forecast

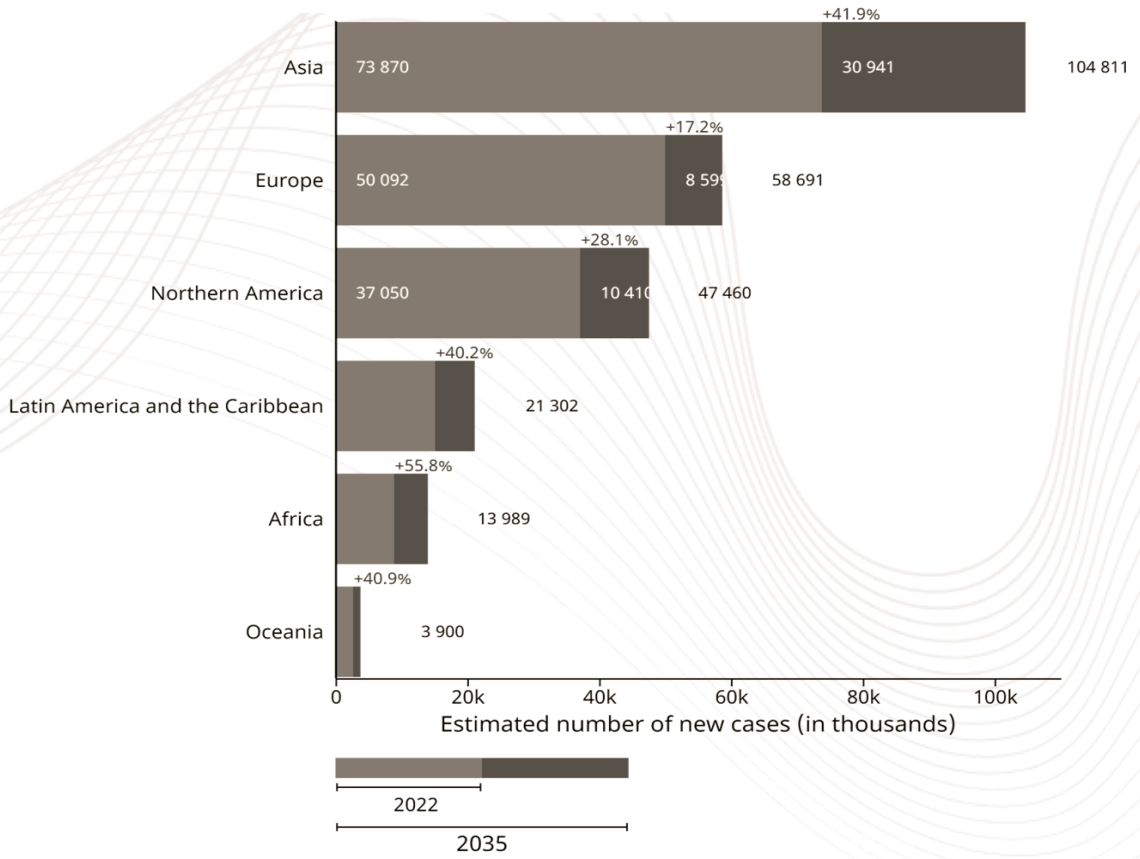


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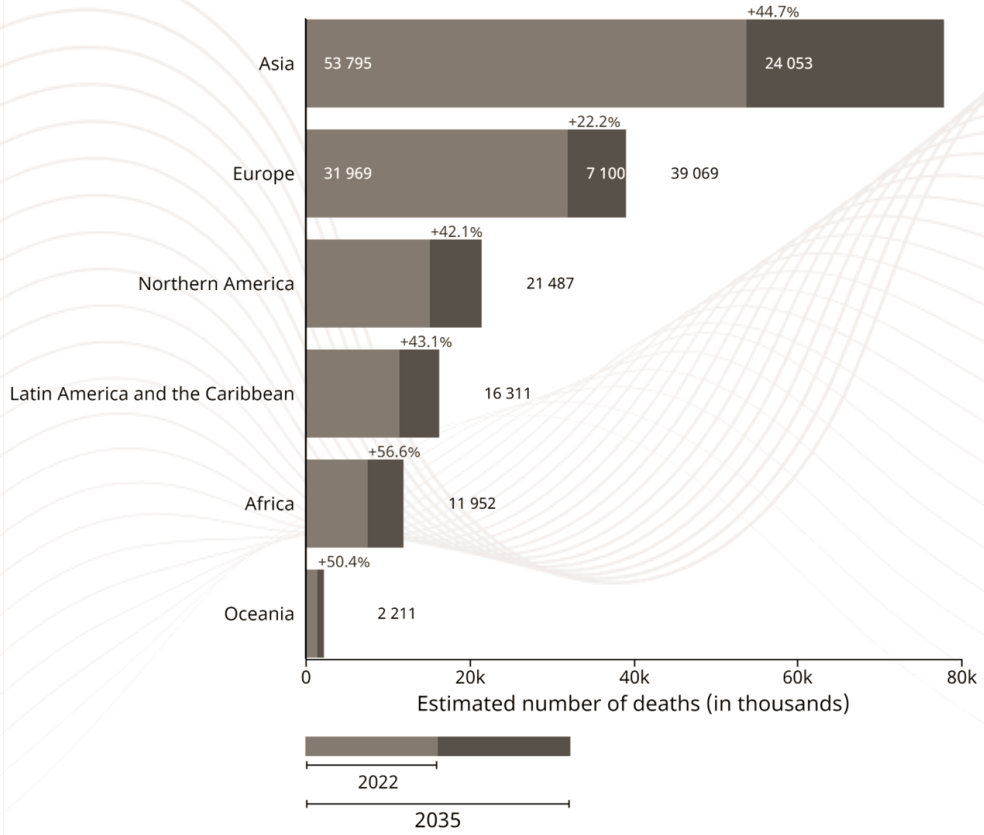
Multiple Myeloma Stats

Estimated from 2022-2035, Male-female, age-0-85+

INCIDENCE (no. of new cases)



MORTALITY (no. of death)



International Agency for Research on Cancer
World Health Organization
GLOBAL CANCER OBSERVATORY

Sources: WHO

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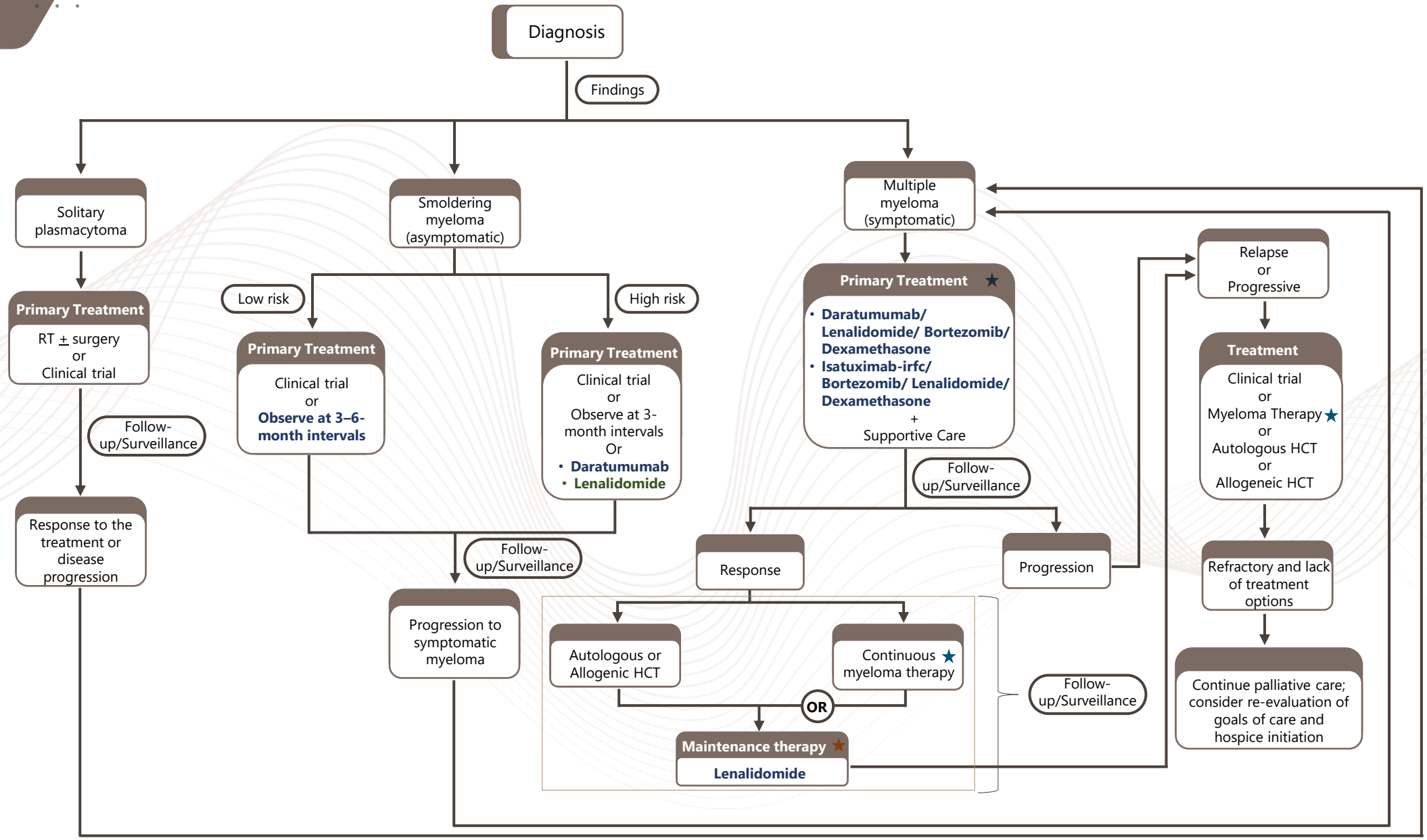
Regulatory Updates

Policies, Approvals and Guidelines

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NCCN: Treatment Algorithm



★ **Other Recommended**
- **Bortezomib/Lenalidomide/ Dexamethasone**
- Carfilzomib/Lenalidomide/ Dexamethasone
- Daratumumab/Carfilzomib/ Lenalidomide/Dexamethasone
- Isatuximab-irfc/Carfilzomib/ Lenalidomide/Dexamethasone

Useful in Certain Circumstances
- Bortezomib/Cyclophosphamide/ Dexamethasone
- Carfilzomib/Cyclophosphamide/ Dexamethasone
- Daratumumab/Bortezomib/ Cyclophosphamide/Dexamethasone
- Dexamethasone/Thalidomide/ Cisplatin/Doxorubicin/ Cyclophosphamide/Etoposide/ Bortezomib (VTD-PACE)

★ **Other Recommended**
- Carfilzomib/Lenalidomide
- Daratumumab/Lenalidomide

Useful in Certain Circumstances
- Bortezomib ± Lenalidomide
- Daratumumab
- **Ixazomib**

★ Refer to the primary treatment of multiple myeloma

Category 1

Category 2B

NOTE: All recommendations are category 2A unless otherwise indicated.

Source: NCCN Guideline Version 4.2026

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Approval Timeline

2015-2017

- 2015 / 2016 — DARZALEX (daratumumab) — Johnson & Johnson
- 2015 / 2016 — Empliciti (elotuzumab) — Bristol Myers Squibb
- 2015 / 2016 — NINLARO (ixazomib) capsules — Takeda
- 2015 / 2016 — XPOVIO selinexor tablets — Karyopharm Therapeutics; NEXPOVIO (selinexor) tablets — Stemline, A Menarini Group Company

2018-2020

- 2020 / 2020 — SARCLISA (isatuximab-irfc) — sanofi
- 2020 / 2025 — BLENREP belantamab mafodotin-blmf for injection 70 mg — GSK
- 2020 / 2025 — DARZALEX Faspro (daratumumab and hyaluronidase-fihj) — Johnson & Johnson

2021-2023

- 2021 / 2021 — Abecma (idecabtagene vicleucel) — Bristol Myers Squibb
- 2022 / 2022 — CARVYKTI (ciltacabtagene autoleucel) — Johnson & Johnson
- 2022 / 2022 — TECVAYLI (teclistamab-cqyv) — Johnson & Johnson
- 2023 / 2023 — ELREXFIO (elranatamab-bcmm) — Pfizer
- 2023 / 2023 — TALVEY (talquetamab-tgvs) — Johnson & Johnson

2024-2026

- 2025 / 2025 — LYNOZYFIC (linvoseltamab-gcpt) — REGENERON

Mechanism of Action
BCMA-directed
GPRC5D-directed
CD38-directed
SLAMF7-directed
Nuclear export inhibitor
Proteasome inhibitor

Key Outcomes

- ✓ J&J dominates the multiple myeloma market, leading with a broad and frequently approved portfolio
- ✓ Rapid innovation in myeloma therapy with steadily increasing numbers of approvals and mechanisms over the last decade

Oral | Intravenous | Subcutaneous

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2025 : Designation Highlights (1/2)

Designation	Orphan Drug Designation	Fast Track Designation	Accelerated Approval
Regulatory	U.S. FOOD & DRUG ADMINISTRATION	U.S. FOOD & DRUG ADMINISTRATION	U.S. FOOD & DRUG ADMINISTRATION
Date of Designation	February 12, 2025	May 05, 2025	July 2, 2025
Company Name	OPNA BIO	ICHNOS GLENMARK INNOVATION	REGENERON
Molecule Name	**OPN-6602**	**ISB 2001**	**LYNOZYFIC (linvoseltamab-gcpt)**
Mechanism of Action	Dual CBP/EP300 BRD inhibitor	Tri-specific antibody (Targets BCMA and CD38 on myeloma cells and CD3 on T cells)	BCMAxCD3 bispecific antibody
Note: Additional information.	**Note:** Currently assessing in Phase 1 (NCT06433947) at multiple sites in the U.S. and expects to complete in 2026.	**Note:** Currently assessing in Phase 1 (NCT05862012) at 9 sites in U.S. and Australia.	**Note:** AA is based on Phase 1/2 (NCT03761108) results. Also, received conditional marketing approval by EMA (Apr, 2025).
Sources	**Opna Bio PR Feb'25**	**IGI PR May'25**	**Regeneron PR Jul'25**

Modality (M) Small Molecules Monoclonal Antibody Cell Therapy Antibody Drug Conjugates

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2025 : Designation Highlights (2/2)

Orphan Drug Designation

U.S. FOOD & DRUG ADMINISTRATION

July 2, 2025

BrightPath Biotherapeutics

BP2202

CAR-T Therapy

Note: Planning to initiate clinical trial in U.S.

BrightPath PR Jul'25

Fast Track Designation

U.S. FOOD & DRUG ADMINISTRATION

October 23, 2025

Heidelberg PHARMA
Focused Cancer Therapies

HDP-101
(pamlectabart tismanitin)
Anti-BCMA

Note: Based on non-clinical as well as clinical data from ongoing Phase 1/2a (NCT04879043) data.

Heidelberg Pharma PR Oct'25

Breakthrough Therapy Designation

U.S. FOOD & DRUG ADMINISTRATION

December 09, 2025

Johnson & Johnson

TECVAYLI + DARZALEX FASPRO

BCMA Bi-specific antibody + Anti-CD38, endoglycosidase

Note: First bi-specific combination. Based on Phase 3 (NCT05083169) results.

J&J PR Dec'25

CNPV*

U.S. FOOD & DRUG ADMINISTRATION

December 15, 2025

Johnson & Johnson

TECVAYLI + DARZALEX FASPRO

BCMA Bi-specific antibody + Anti-CD38, endoglycosidase

Note: First bi-specific combination. Based on Phase 3 (NCT05083169) results.

J&J PR Dec'25

Modality (M) Small Molecules Monoclonal Antibody Cell Therapy Antibody Drug Conjugates

* Commissioner's National Priority Voucher (CNPV) Pilot Program: Detailed information is available in the Regulatory Update slide.

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Regulatory Policy Updates

Commissioner's National Priority Voucher (CNPV) Pilot Program

Accelerating Critical Drug and Biologic Reviews for Companies support

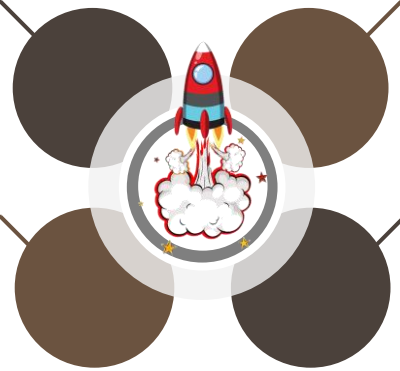
What is the CNPV Pilot Program?

Launched in **June 2025** by the **US FDA**, this groundbreaking initiative dramatically transforms regulatory timelines and processes for medicines aligned with national priorities.

Benefits:

Revolutionary Speed
Reduces review times from 10-12 months to just 1-2 months

Broad Scope
Applies to both prescription drugs and biologics meeting U.S. national priorities



Enhanced Communication
Direct access to FDA experts with rolling review capabilities throughout development

Rapid Decisions
Review outcomes delivered within 30 to 60 days after application submission

Potential for accelerated approval if applicable requirements are met

CNPV	VS	Other designation
Offers 1-2 months	Review Time	**Priority Review** 6 months (for serious conditions)
Adds rapid decision timelines	Interaction	**Fast Track** Frequently
Provides ultra-fast review	Guidance	**Breakthrough Therapy** Intensive guidance for exceptional promise

CNPV specific criteria:

- Public health crises in U.S.
- Transformative cures for American
- Large unmet needs
- U.S. manufacturing & trials
- Affordability

Sources: FDA CNPV

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Highlights of CNVP

Awarded National Priority Vouchers

- 1st batch: 9 Vouchers — **Oct'25**
- 2nd batch: 6 Vouchers

Nov'25

- **Zongertinib** for HER2 lung cancer
- **Bedaquiline** for drug-resistant tuberculosis in young children
- **Dostarlimab** for rectal cancer
- **Casgevy** for sickle cell disease
- **Orforglipron** for obesity and related health conditions
- **Wegovy** for obesity and related health conditions

- FDA awarded NPV to Teclistamab in combination with daratumumab based on Phase 3 study results for r/r multiple myeloma

- 2 more NPV vouchers in 2025

Dec'25

- **Enlicitide decanoate** for lowering LDL cholesterol
- **Sacituzumab Tirumotecan** is a trophoblast cell-surface antigen 2 (TROP2)-directed antibody-drug conjugate

- **Pergoveris** for infertility
- **Teplizumab** for Type I diabetes
- **Cytisinicline** for nicotine vaping addiction
- **DB-OTO** for deafness
- **Cenegermin-bkbj** for blindness
- **RMC-6236** for pancreatic cancer
- **Bitopertin** for porphyria
- **Ketamine** for domestic manufacturing of a critical drug for general anesthesia
- **Augmentin XR** for domestic manufacturing of a common antibiotic

1st Approval under CNVP

FDA NEWS RELEASE

FDA Proactively Awards National Priority Voucher Based on Strong Phase 3 Study Results

For Immediate Release: December 15, 2025

The U.S. Food and Drug Administration today awarded a national priority voucher to teclistamab in combination with daratumumab for relapsed/refractory multiple myeloma. This brings the total number of products receiving an award under the Commissioner's National Priority Voucher pilot program to 16.

"We're on a mission to deliver more cures and meaningful treatments to the American people. This means proactively identifying potentially transformative therapies," **said FDA Commissioner Marty Makary, M.D., M.P.H.** "Within hours of the trial results being published in the American Society of Hematology conference program, FDA leaders read the study, consulted with internal experts, and the following day contacted the company to discuss a national priority voucher. When a treatment demonstrates outstanding trial results, we have a duty to patients to move swiftly."

In Phase 3 clinical trial data released November 24, Tec-Dara showed significant improvements over the standard of care in both progression-free survival and overall survival among patients previously treated with one to three prior lines of therapy. At three years, more than 80% of Tec-Dara recipients remained free of progression. These results were also published in the New England Journal of Medicine on December 9.

FDA NEWS RELEASE

First Approval in Commissioner's National Priority Voucher Pilot Program Strengthens Domestic Antibiotic Manufacturing Capacity

For Immediate Release: December 09, 2025

This press release was updated on 12/10/25 to include the name of the sponsor.

The U.S. Food and Drug Administration today approved Augmentin XR (amoxicillin-clavulanate potassium) under the Commissioner's National Priority Voucher (CNPV) pilot program, marking the first approval achieved through this review pathway. The approval was completed in just two months, representing a major reduction of the review timeline for this type of application.

"Over the last few decades, America lost control of supply chains for key medicines we depend on. That chapter is over – we're entering a new era of manufacturing here at home," **said FDA Commissioner Marty Makary, M.D., M.P.H.** "This first drug approval under the CNPV pilot program will strengthen domestic manufacturing and increase our national security."

The Augmentin XR application demonstrated clear alignment with the CNPV program's national health priorities by strengthening the U.S. drug supply chain through enhanced domestic manufacturing capacity at a U.S. facility. This approval will also help address antibiotic shortages in the U.S. that have plagued the healthcare system over the past two decades. This approval was granted to USAntibiotics.

FDA U.S. FOOD & DRUG ADMINISTRATION

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Landscape

Beyond ASH



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INSIGHT CORNER

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Landscape (1/3)

USA Europe China Global ROW *i*: induction therapy *c*: consolidation therapy

1L/1L+

PCD	2025-2030	2031-2036
Phase 3	**Iberdomide + Isatuximab** BMS IKZF1&3 degrader **BLENREP + REVLIMID + DEX** GSK BCMA inhibitor **SCTC21C + VRd** Sinocelltech CD38 inhibitor	**Blenrep + Velcade + VRd** GSK BCMA inhibitor **LYNOZYFIC + DRd** Regeneron BCMA x CD3
Phase 2	**LYNOZYFIC** Regeneron BCMA x CD3 **DVRd*i* + Cilta-cel + Tal-D*c* ±Tec-D*c*** Allorunning BCMA modulator **Cilta-cel + CTx + *i*** Allorunning BCMA/CD19 modulator	**LYNOZYFIC + REVLIMID** Regeneron BCMA x CD3 **Smoldering Multiple Myeloma** **LYNOZYFIC** Regeneron BCMA x CD3 **LYNOZYFIC** Regeneron BCMA x CD3
Phase 1	**TQB2029*** Chia Tai Tianqing CD3 x GPRC5D **Ramantamig + POMALYST/DARZALEX (+ REVLIMID)** J&J BCMA x CD3 x GPRC5D **RN1201** Allorunning BCMA/CD19 modulator **SHR-9539*** Suncadia Biopharma CD3 stimulant **CT071** CARsgen GPRC5D inhibitor	

Modality-> Biologics Protein ADC CGT Small Molecules

Route of Administration: Oral, Intravenous, Intramuscular, Subcutaneous

Combination / Monotherapy

CTx: Chemotherapy
DEX: Dexamethasone
*administered as injection

This presentation is non-exhaustive. For additional details or clarifications, please feel free to contact us at bd@octavusconsulting.com

INSIGHT CORNER

Landscape (2/3)

USA Europe China Global ROW *i*: induction therapy *c*: consolidation therapy

R/R

PCD	2025-2030	2031-2036
Phase 3	**Elranatamab** Pfizer BCMA x CD3	
	TALVEY + POMALYST/ TECVAYLI J&J CD3 x GPRC5D	
	SG301 + POMALYST + DEX Hangzhou Sumgen CD38 inhibitor	
	Equecabtagene Autoleucel Nanjing IASO BCMA inhibitor	
	Linvoseltamab + Carfilzomib Regeneron BCMA x CD3	
	Anitocabtagene autoleucel Allorunning BCMA inhibitor	
	Etentamig AbbVie BCMA x CD3	
	QLS32015 Qilu CD3 x GPRC5D	
	CM336* Keymed Biosciences BCMA x CD3	
	Ramantamig J&J BCMA x CD3 x GPRC5D	
	Arlo-cel BMS GPRC5D inhibitor	
Phase 2	**MBS314** Beijing Mabworks BCMA x CD3 x GPRC5D	
	Purinostat Mesylate + POMALYST + DEX Chengdu Zenitar HDAC1&2 inhibitor	
	Ramantamig J&J BCMA x CD3 x GPRC5D	
	Arlocabtagene autoleucel ((Arlo-cel) BMS GPRC5D inhibitor	
	CT071 CARsgen GPRC5D inhibitor	
	LYNOZYFIC Regeneron BCMA x CD3	
	F182112 + DAR Shandong New Time BCMA x CD3	
	BCD-248 Biocad BCMA x CD3	
	CM313 Allorunning CD38 inhibitor	
	QLS32015 + POMALYST + CTx + REVLIMID/ QL2109/ DARZALEX Qilu CD3 x GPRC5D	
	mRNA-2808 ModernaTX BCMA, FCRL5 & GPRC5D inhibitor	
	Cemsidomide + DEX C4 Therapeutics IKZF1&3 degrader	
	BLENREP + DEX + VELCADE/ POMALYST/ KYPROLIS GSK BCMA inhibitor	
	Inobrodib + POMALYST + DEX CellCentric EP300 & p300-CBP inhibitor	
	SARCLISA + KYPROLIS + DEX Sanofi CD38 inhibitor	
	HuL-001 HuniLife ENO1 inhibitor	**REGN7945 + LYNOZYFIC** Regeneron CD28 & 38 modulator
	ABBV-453 AbbVie Bcl-2 inhibitor	**IBI3003** Innovent Biologics BCMA x CD3 x GPRC5D
	KQ-2003 Novatim Immune BCMA/CD19 modulator	**Velinotamig*** Allorunning BCMA x CD3
	BT02 Biotroy ITPRIPL1 inhibitor	**TALVEY + CARVYKTI** J&J CD3 x GPRC5D + BCMA modulator
		Etentamig + Iberdomide AbbVie BCMA & CD3

Modality-> Biologics Protein ADC CGT Small Molecules

Route of Administration: Oral, Intravenous, Intramuscular, Subcutaneous

Combination / Monotherapy

CTx: Chemotherapy
DEX: Dexamethasone
*administered as injection

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INSIGHT CORNER

Landscape (3/3)

USA Europe China Global ROW *i*: induction therapy *c*: consolidation therapy

R/R

PCD 2025-2030

Phase 1

CT0596 CARsgen BCMA inhibitor	**ESO-T01** EsoBiotec BCMA modulator	**BL-M24D1** Sichuan Baili Unknown	**BG2301** Guangzhou Bio-gene BCMA & GPRC5D inhibitor	**BMS-986453** BMS BCMA & GPRC5D inhibitor	**Meta10-BCMA** Leman Biotech BCMA modulator	
TQB2934 Chia Tai Tianqing BCMA x CD3	**LVIVO-TaVec200** Nanjing Legend GPRC5D inhibitor	**RN1101** Allorunning BCMA/CD19 modulator	**SIM0500** Jiangsu Simcere BCMA x CD3 x GPRC5D	**OriCAR-017** OriCell GPRC5D inhibitor	**Invivo CAR-T** Chongqing Precision CD19 inhibitor	**QXL138AM** Nammi Therapeutics CD138 inhibitor & IFNα2 stimulant
YKST02 Excyte Biopharma BCMA x CD3	**CG-105** Cells & Genes Biotech BCMA inhibitor	**QLS4131** Qilu BCMA x CD3 x GPRC5D	**OriV508** Allorunning BCMA/CD19 modulator	**SL4903** Hebei Senlang Unknown	**NT-I7#** NeoImmuneTech IL-7Rα agonist	**Arlo-cel + ALNUC** **MEZI /Iberdomide or ELREXFIO** BMS GPRC5D inhibitor
LMY-920 Luminary BAFF-R, BCMA inhibitor & TACI modulator	**Etentamig** AbbVie BCMA x CD3	**KLN-1010** Kelonia BCMA modulator	**Ramantamig + DARZALEX** J&J BCMA x CD3 x GPRC5D	**SAR446523** Sanofi GPRC5D inhibitor	**REVO-UWD-00B** Wondercel Biotech BCMA modulator	**OPN-6602 + DEX** Opna Bio CPB & EP300 inhibitor
Cemsidomide + Elranatamab C4 Therapeutics IKZF1 & 3 degrader	**CBG002*** Carbiogene BCMA inhibitor	**JNJ-87562761** J&J GPRC5D inhibitor	**ELREXFIO + Iberdomide** Pfizer BCMA x CD3	**CAR19-BCMA dual-target CAR-T** Hebei Taihe Chunyu CD19 modulator	**Cevostamab** Allorunning CD3 stimulant & FCRL5 inhibitor	**BZE2204** Shanghai Cell Therapy BCMA, CD19 & CD22 inhibitor

Modality-> Biologics Protein

ADC CGT Small Molecules

Route of Administration

Oral Intravenous Intramuscular Subcutaneous

Combination

Monotherapy

CTx: Chemotherapy
DEX: Dexamethasone
*administered as injection

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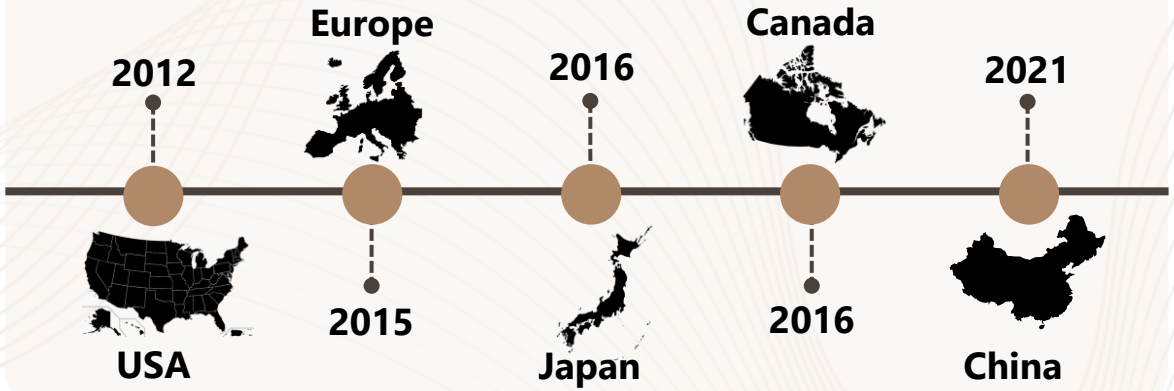
Product Profile: Carfilzomib

Kyprolis® (carfilzomib) for Injection

Mechanism of Action	Proteasome inhibitor
Route of Administration	Intravenous
Approved dose	Lyophilized powder:10 mg, 30 mg or 60 mg (single-dose vial)

AMGEN

Global Approval Timeline



Octavus Insights

- ✓ KYPROLIS in combo with lenalidomide and dexamethasone, is already approved for r/r multiple myeloma and is now being studied in phase 3 study to expand its use and enable earlier access in patients with newly diagnosed multiple myeloma
- ✓ In ASPIRE, mPFS of 26.3 mos and mOS of 48.3 mos highlight durable disease control and meaningful long-term benefit with this combination
- ✓ Triplet shows strong anti-myeloma activity, with overall response rates of 87% in the ASPIRE trial and 94% in the COBRA trial, underscoring how reliably patients respond to this combination

Key Results

Regimen		**Carfilzomib (KYPROLIS) + Lenalidomide + Dexamethasone**	
Line of therapy		Relapsed or Refractory Multiple Myeloma	Newly diagnosed Multiple Myeloma#
Trial ID		ASPIRE (NCT01080391)	COBRA (NCT03729804)
Phase		Phase 3	
Efficacy	**n**	396	126
	mFU (mos)	67	35
	mPFS (mos)	26.3	NR
	mOS (mos)	48.3	-
	ORR (%)	87	94
	mDOR (mos)	28.6*	-
	mTR (mos)	1	-
Source		KYPROLIS US Label	ASH 2025

interim analysis (not approved yet), *in 345 pts

Abb.: mFU: median follow-up; mPFS: median progression-free survival; mOS: median overall survival; ORR: overall response rate; mDOR: median duration of response; mTR: median time to response; NR: Not-reported

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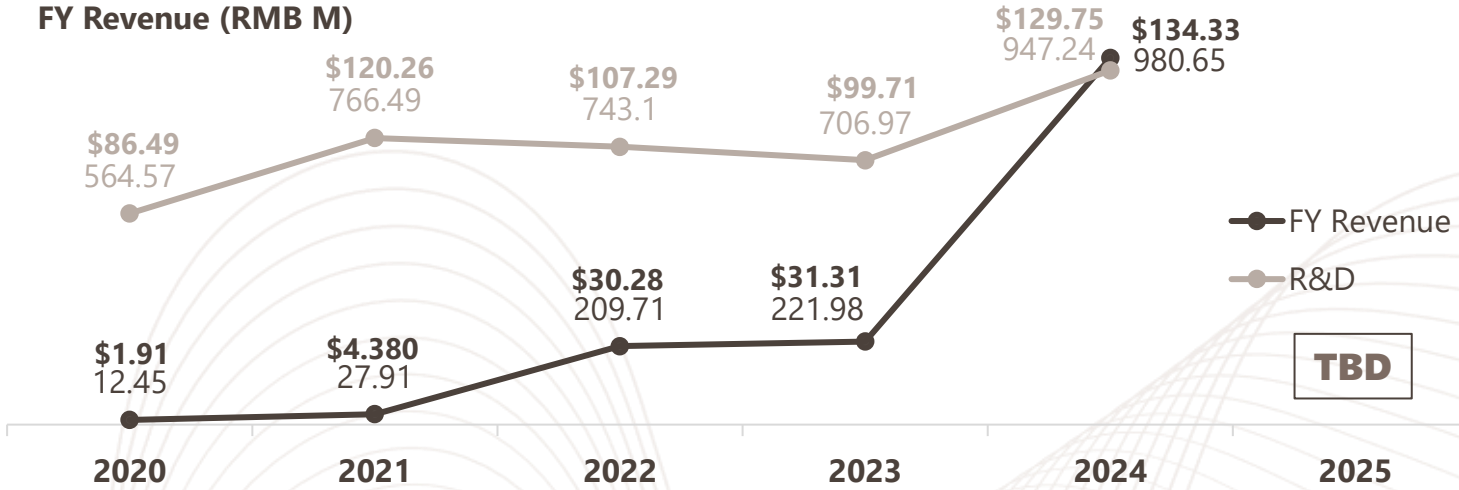
Company Profile: Ascentage Pharma

Overview

Ascentage Pharma is a global, integrated biopharmaceutical company focused on discovering, developing, and commercializing therapies that address unmet medical needs in cancer, primarily hematological malignancies, and is currently conducting more than 40 clinical trials across selected regions worldwide

- **Headquarter** China
- **Founded Year** 2009
- **Stock Market**
  - HKEX: 6855 (Oct'19)
  - NASDAQ: AAPG (Jan'25)
- **CEO** Dr. Dajun Yang

Financials



- **1H'25 Vs 1H'24** $32.6 M Vs $113.4 M
- **1H'25: Olverembatini** RMB 217.4 M ($30.3)

Haematological Pipeline

Drug candidate	Indication	Phase 2	Phase 3
Olverembatini (HQP 1351)	CML, Ph+ ALL, SDH-deficient GIST		
Lisaftoclax (APG-2575)	CLL/SLL, AML, MDS, MM		
Alrizomadlin (APG-115)	ACC, MPNST, AML, MDS, Pediatric solid tumor		

r/r MM clinical programs

- Lisaftoclax ± Lenalidomide and Dexamethasone — NCT04674514
- Lisaftoclax ± Novel Therapeutic Regimens — NCT04942067

Abb.: CML: chronic myeloid leukemia; Ph+ ALL: philadelphia chromosome-positive acute lymphoblastic leukemia; CLL/SLL: chronic lymphocytic leukemia/ small lymphocytic lymphoma; AML: acute myeloid leukemia; MDS: myelodysplastic syndrome; r/r MM: relapsed/ refractory multiple myeloma I **Sources:** Ascentage Pipeline; Ascentage AR'24; Ascentage Linkedin

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INSIGHT CORNER

News Analysis

C4 THERAPEUTICS Signs Collaboration and Supply Agreement with PFIZER for Multiple Myeloma

To access C4T's oral degrader in combination with dexamethasone and BCMAxCD3 bispecific antibody



Highlights:

- C4 Therapeutics will initiate a Phase 1b study of cemsidomide + dexamethasone + elranatamab as ≥2L therapy for patients with multiple myeloma
- Pfizer will provide elranatamab at no cost, while C4T sponsors and conducts the trial

Upcoming milestones:

- Planning to initiate 2 clinical trials for further evaluation:
  - Phase 2 registrational trial of cemsidomide + dexamethasone expected to start in the 1Q'26
  - Phase 1b trial of cemsidomide + dexamethasone + elranatamab expected to start in the 2Q'26
- If the combination confirms strong T-cell activation and enhanced anti-myeloma responses, C4T plans to advance the regimen into broader development, aiming to improve outcomes in earlier lines of therapy

Key Outcome:

- The agreement secures free supply of elranatamab for C4T's Phase 1b study, enabling evaluation of a novel combination regimen without bearing the expense or risk of securing the bispecific antibody independently
- The trial is designed to define an optimal dose of cemsidomide + elranatamab while characterizing safety and tolerability in r/r multiple myeloma

Sources: C4 Therapeutics PR Oct'25

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Investment

MM deals 2020-2025



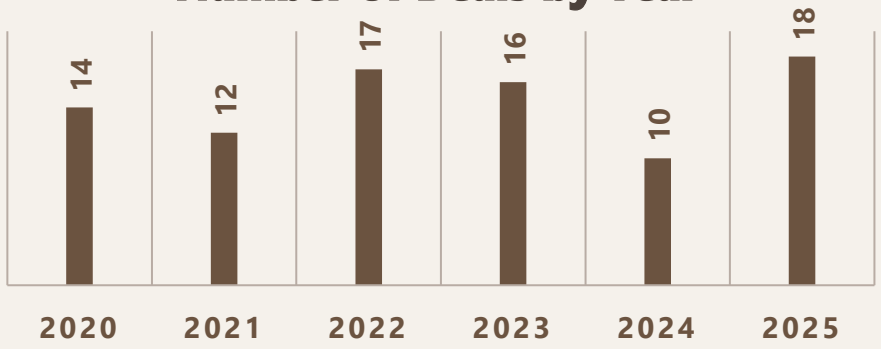
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Deal dashboard

2020 – 2025 (till Dec'25)

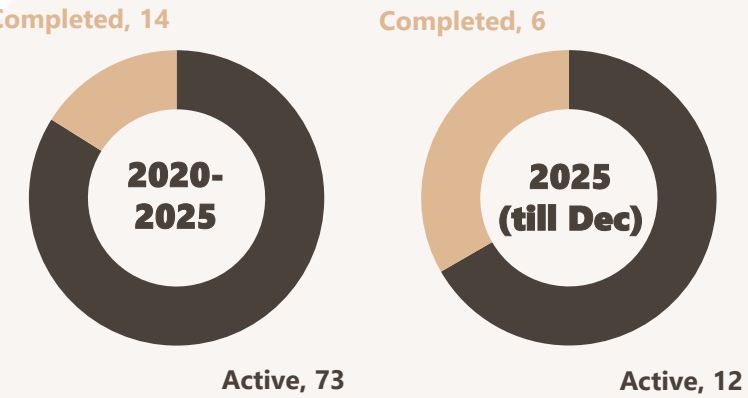
Number of Deals by Year



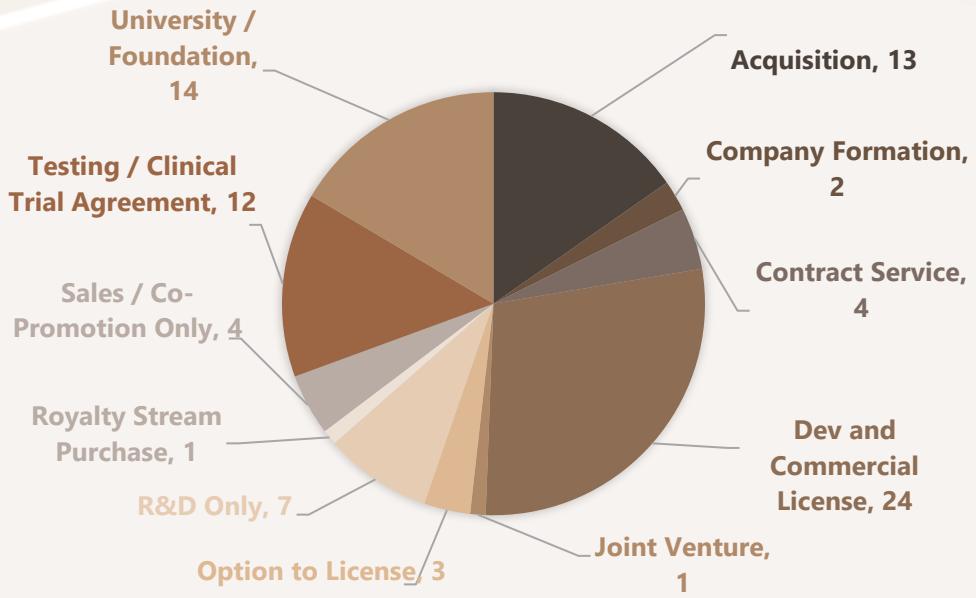
Distribution by Stage Type

Stages	No. of deals
Platform/ Discovery	20
Preclinical / IND	13
Phase 1	29
Phase 2	9
Phase 3	6
Approved	9

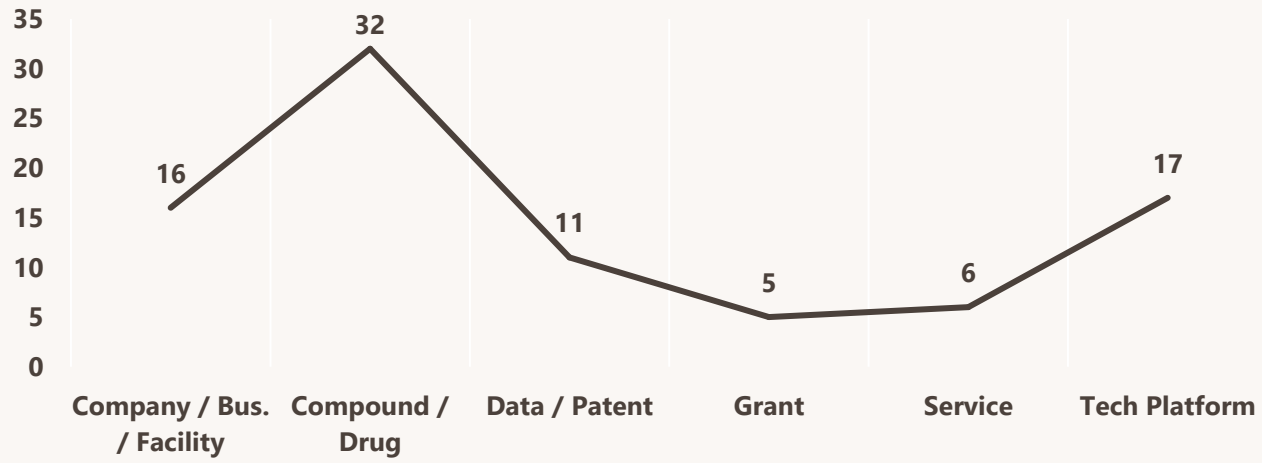
Deals Status



Type of Deals



Deals Asset Type



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Deal Analysis

Tempest Therapeutics to Acquire Factor Biosciences' Parallel Dual CAR-T TPST-2003 for Multiple Myeloma

Nov 19, 2025

Overview:

- Tempest Therapeutics has entered into a definitive agreement to acquire TPST-2003, a parallel-structured dual CD19/BCMA CAR-T therapy, from Factor Biosciences
- Transaction is expected to close in early 2026
- Existing cash at closing, along with an investment commitment from Factor, is expected to fund operations through mid-2027, covering key development and data milestones in 2026–2027

Program Highlights:

- TPST-2003 Dual CD19/BCMA CAR-T program for r/r multiple myeloma
- Phase 1 study completed in r/r MM patients; data readout expected in 2026
- China BLA submission planned for 2027

Geographic & Development Rights:

- Tempest will hold global rights to TPST-2003 outside China, India, Turkey, and Russia
- Potential pivotal registrational study in US for rrMM planned for 2027
- Data from the Chinese pivotal study is expected to support probability of success and may be referenced for the planned China BLA

Key Transaction Terms:

- Tempest will issue 8,268,495 shares of common stock (par value $0.001) to a Factor affiliate, representing ~65% of outstanding shares (including newly issued shares) as of November 19, 2025
- Existing Tempest shareholders will receive 1 common stock warrant per outstanding share prior to closing
  - Exercise price: $18.48
  - Term: 5 years from issuance
  - Exercisable immediately

Leadership Update:

- Upon closing, Matt Angel, PhD, will become President and CEO of Tempest Therapeutics, whereas Current President and CEO Stephen Brady will transition to Chairman of the Board

Key Outcomes:

- This deal will diversify Tempest's pipeline and extend the Company's runway to mid 2027, potentially through value-creating milestones

Sources: Tempest PR Nov'25

INSIGHT CORNER

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Social Media Analysis

Engagement Overview

Exploring how multiple myeloma conversations shape the digital footprint and community impact

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KOL PROFILE DASHBOARD



Joseph Mikhael

INSTITUTION
Translational Genomics Research Institute (TGen)

DEPARTMENT
Multiple Myeloma

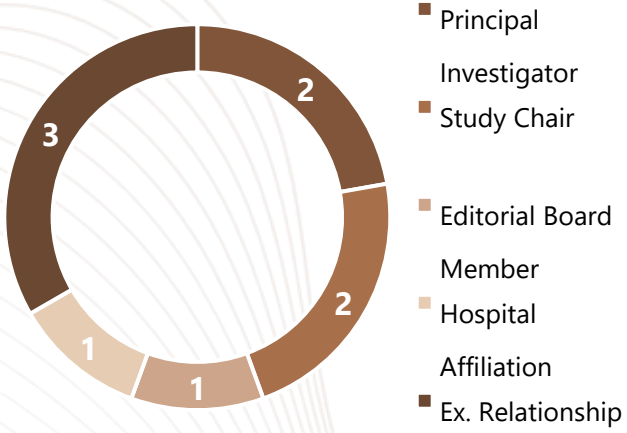
SPECIALITY
Hematology

EMAIL
jmikhael@myeloma.org

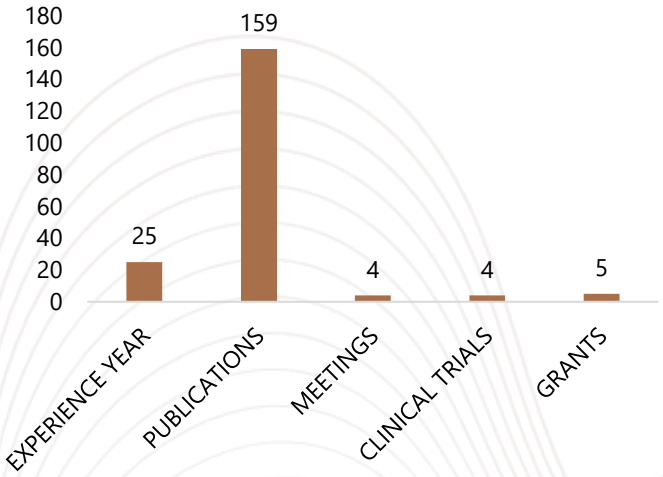
LOCATION
Phoenix, AZ, USA

MEMBERSHIPS	RECORDS	EX. RELATIONS	PUBLICATIONS	CLINICAL TRIALS
5	5	18	159	4

Memberships



Records



Ex. Relationships

INTERNATIONAL MYELOMA FOUNDATION

35 YEAR ANNIVERSARY

HONORHEALTH™

Publications

- **Review:** Current and future role of carfilzomib-based quadruplet combinations as therapy for newly diagnosed multiple myeloma: ***Hemasphere***: 2025 Jul 16;9(7):e70178.
- **Clinical Trial:** Isatuximab plus carfilzomib-dexamethasone versus carfilzomib-dexamethasone in patients with relapsed multiple myeloma (IKEMA): ***Lancet Haematol:*** 2024 Oct;11(10):e741-e750.
- **Editorial:** How can we stamp out high-risk myeloma?: ***Blood:*** 2024 May 16;143(20):2015-2016.

Clinical Trials

DRUG	INDICATION	PHASE	SPONSOR
Plerixafor, Lenalidomide	Multiple Myeloma	Phase 2	Mayo Clinic
Cyclophosphamide, Carfilzomib, Thalidomide, and Dexamethasone	Multiple Myeloma	Phase 1/2	Mayo Clinic
Pomalidomide, Bortezomib, and Dexamethasone	Multiple Myeloma	Phase 1/2	Mayo Clinic

NOTE: Joseph Mikhael has total 4 clinical trial according to clinical trial.gov in which 3 trails are complete and 1 has been terminated

INSIGHT CORNER

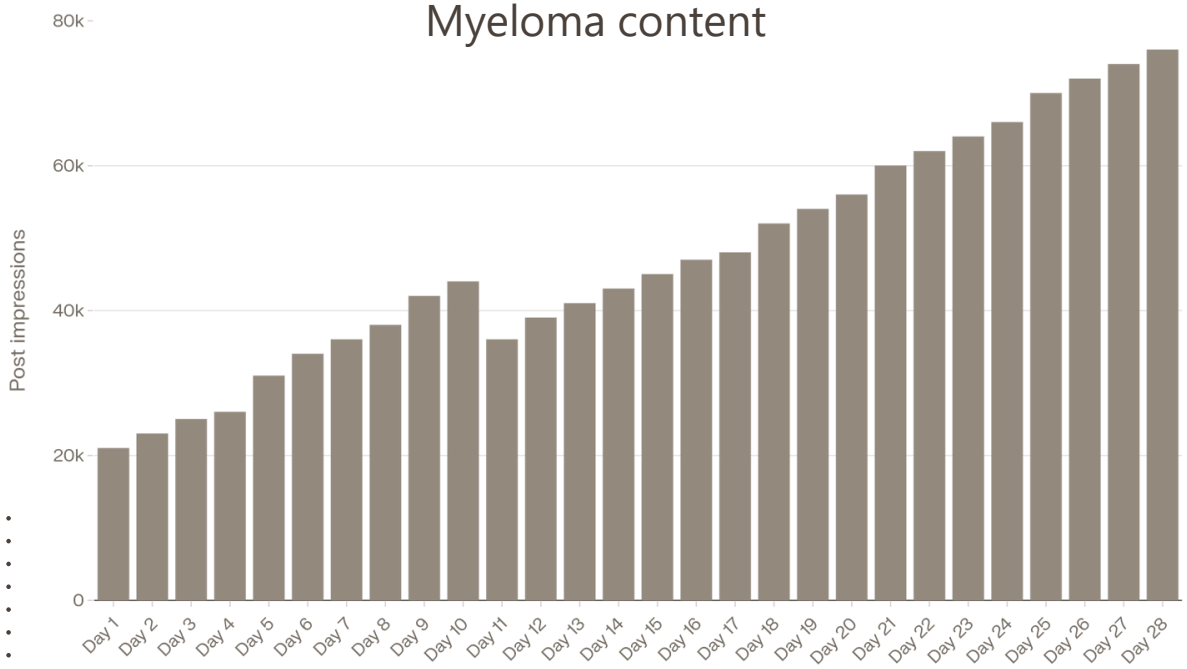
This presentation is non-exhaustive. For additional details or clarifications, please feel free to contact us at bd@octavusconsulting.com

LinkedIn Impact Highlights

960K+

Post Impressions

Total reach in the last 28 days for ASH 2025 Multiple Myeloma content



70+

Engagement Ratio

Impressions to likes ratio indicating viral reach within professional networks

Engagement is highest for actionable, education-focused posts with strong save and comment rates, supported by hashtag strategies (#ASH25, #MultipleMyeloma) that broaden reach beyond core followers worldwide.

INSIGHT CORNER

LinkedIn Highlights



Leon van Wouwe · 3rd

Versatile leader in clinical development operations, building the bridges for wellbeing, between biotech, health tech, clinical researchers, clinicians and patients. Views expressed are my own.

At #ASH Annual Meeting in Orlando this week, it is obvious that research in multiple myeloma continues to progress rapidly, generating a wealth of new insights, but also raising more questions.

For example, Johnson & Johnson presented new overall survival data demonstrating Tecvayli, when combined with Darzalex, cut the risk of death by 54%. Good news for patients for sure, but raising questions about whether clinicians should give multiple myeloma patients antibody treatment before #CAR-T therapy or after. [1]

Physicians aren't sure how to answer that question, or if getting one therapy would preclude patients getting a subsequent therapy. Current data shows that bispecific antibody targeting regimens tend to work better when administered after CAR-T treatment, compared to the reverse - as pointed out by Michael Rosenzweig of the multiple myeloma research group at the City of Hope hospital institute. Besides strictly medical considerations, patients may have their own preferences to. For instance being off-treatment for long periods as with CAR-T`s may appeal more to patients. [1]

Other than CAR-T and bispecific antibody therapies, there are new multiple myeloma regimens in the pipeline, for instance with companies like Bristol Myers Squibb who are running a Phase III program with #CELMoDs, a new type of protein degrader. [2]

Reflective Questions:

1) How can we build the bridges between computer data science, clinical care and the pharmaceutical industry to drive innovation in earlier diagnosis of multiple myeloma?

2) How can we scale the deployment of ai models that can detect the patters in EMR data that indicate likely undiagnosed disease, to support HCP in triaging individuals for targeted multiple myeloma testing, in practical and sustainable ways?

Gary Nolan • 3rd+ 2w

Championing education through collaboration in healthcare

Really interesting post; it captures why multiple myeloma feels both exciting and maddening right now. The science is racing ahead, but the system is still catching up.

The CAR-T vs bispecific sequencing debate says it all: incredible data, huge potential, and clinicians saying, "Well, it depends." Hard to blame them when therapy order can shift outcomes, especially once patient preferences; like long treatment-free periods with CAR-T, come into play.

What stands out is that despite these breakthroughs, we're still diagnosing too many patients far too late. Even the most elegant therapy can't help if someone arrives at stage III.

On your questions: the bridge between data science, frontline care, and industry starts with agreeing on the problem. It's not a tech issue; it's a people issue. When clinicians, patient orgs, and industry pull in the same direction, things move.
And scaling AI to spot undiagnosed MM is possible; if it fits workflows, earns trust, and avoids new inequities.

The recipe is simple: make it easy, make it useful, and build it with the people who'll rely on it.

If we get early detection right, that's when CAR-T, bispecifics, CELMoDs, and whatever's next truly transform outcomes not just conference agendas.

Like · 2 | Reply · 1 reply

Leon van Wouwe Author 2w

Versatile leader in clinical development operations, building the bridge...

Gary Nolan thank you, those are very insightful comments you are making and I agree: this is not a tech issue as you say.

Although the tech itself is not trivial, at Volv Global SA we have developed ai models, specifically to detect patterns in EMR that indicate undiagnosed patients for other indications. And deployed these models into clinical workflows, successfully.

The big challenge as you point out as well, is in giving clinicians and others in the healthcare system the reason to believe that the flags are worth their time and attention, their clinical decision making to order testing, or not.

It is all about building the bridges so technology meets human needs, which is what Heather Dombrosky BSN, RN, CRNI was also emphasising in one of her recent posts.

INSIGHT CORNER

Twitter Impact Highlights

500K+

Post Impressions

In which 65% from top 10 threads (e.g., iMMagine-1 CAR-T data, quadruplet therapy updates)

5-10%

Engagement Ratio

(likes, retweets, replies) Viral within professional hematology networks on Twitter

50+

Viral Posts

50+ posts qualified as viral (>5K impressions, >100 interactions):

- Top Myeloma: 12 threads on KarMMa-3, iMMagine-1 CAR-T (Kite/Gilead) – 25K+ views each

Trial recap threads that drive strong retweets and saves deliver the highest engagement, with #ASH25 and #MultipleMyeloma hashtags helping content travel beyond core hematology followers worldwide.

Multiple Myeloma Insights from Twitter

Multiple Myeloma Hub @MM_Hub · Dec 8, 2025

CONGRESS | #ASH25 | POSTER

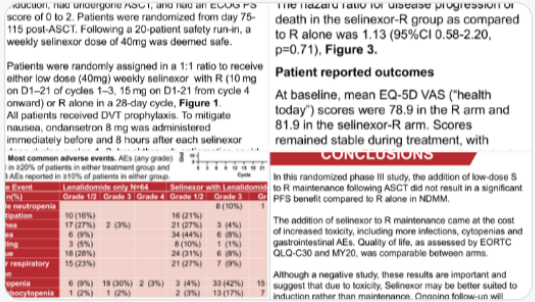
Matthew Rees, St Vincent Hospital, Melbourne, presents findings from the primary analysis of the phase III SEALAND trial of maintenance therapy with selinexor and lenalidomide (SR) vs R alone in patients with NDMM (N = 149).

Thirty-month PFS rates were 65% in the SR group and 69% in the R alone group. QoL assessed by EORTC, QLQ-C30, and MY20 was comparable between both groups. Grade ≥3 SAEs, including GI disorders (12% vs 3%), neutropenia (61% vs 33%), and thrombocytopenia (26% vs 2%), were higher in the SR group compared to R alone group.

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score of 0 to 2. Patients were randomized from day 75-115 post-ASCT. Following a 20-patient safety run-in, a weekly selinexor dose of 40mg was deemed safe.

Patients were randomly assigned in a 1:1 ratio to receive either low dose (40mg) weekly selinexor with R (10 mg on D1–21 of cycles 1–3, 15 mg on D1-21 from cycle 4 onward) or R alone in a 28-day cycle, **Figure 1**.
All patients received DVT prophylaxis. To mitigate nausea, ondansetron 8 mg was administered immediately before and 8 hours after each selinexor

death in the selinexor-R group as compared to R alone was 1.13 (95%CI 0.58-2.20, p=0.71), **Figure 3.**

Patient reported outcomes

At baseline, mean EQ-5D VAS ("health today") scores were 78.9 in the R arm and 81.9 in the selinexor-R arm. Scores remained stable during treatment, with

CONCLUSIONS

In this randomized phase III study, the addition of low-dose S to R maintenance following ASCT did not result in a significant PFS benefit compared to R alone in NDMM.

The addition of selinexor to R maintenance came at the cost of increased toxicity, including more infections, cytopenias and gastrointestinal AEs. Quality of life, as assessed by EORTC QLQ-C30 and MY20, was comparable between arms.

Although a negative study, these results are important and suggest that due to toxicity, Selinexor may be better suited to induction rather than maintenance. Ongoing follow-up will

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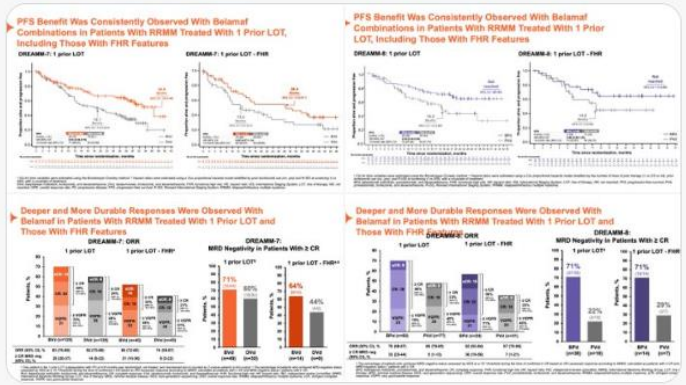
Multiple Myeloma Hub @MM_Hub · Dec 10, 2025

CONGRESS | #ASH2025 | POSTER

María-Victoria Mateos shares findings from a subgroup analysis of patients with functional high-risk RRMM in DREAMM-7 and DREAMM-8, evaluating belamaf-based regimens vs SOC.

In both D7 and D8, belamaf-based regimens (BVd and BPd, respectively) were

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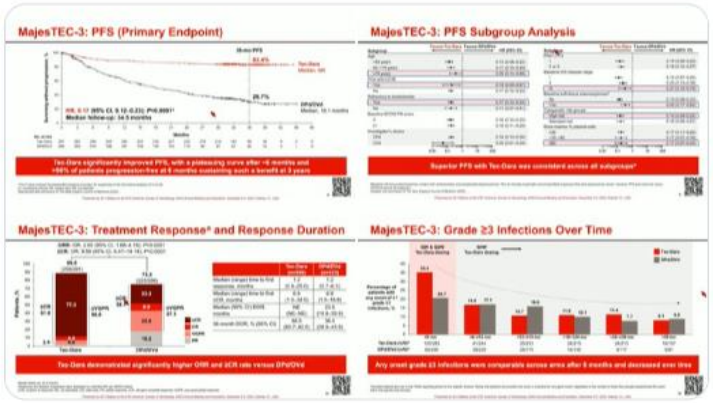
Multiple Myeloma Hub @MM_Hub · Dec 9, 2025

CONGRESS | #ASH25 | PRESENTATION

María-Victoria Mateos shares findings from the phase III MajesTEC-3 study of teclistamab + daratumumab (Tec-Dara) (n = 291) vs investigator's choice of either DPd or DVd (n = 296) for the treatment of RRMM.

Tec-dara resulted in a significantly

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Top views from Twitter highlight:

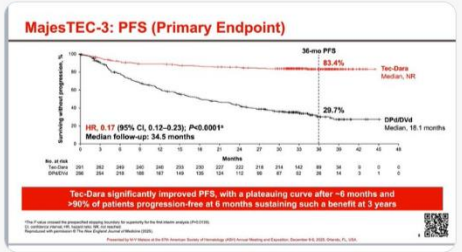
- *Selinexor + lenalidomide (SR) showed comparable efficacy to lenalidomide (R) alone in NDMM, with 30-month PFS rates ≥65% (SR) and 69% (R alone) with manageable safety profile, supporting this as a feasible maintenance option in NDMM*
- *Tec-Dara delivered a striking PFS advantage over DPd/DVd in r/r MM, with 36-month PFS of 83.4% vs 29.7% (HR 0.17), maintained across subgroups, and CRS limited to mostly grd≤2 events supporting Tec-Dara as a strong contender for new SoC in ≥2L myeloma*
- *Belamaf-based regimens (BVd & BPd) deliver longer PFS & deeper, MRD-negative responses than SoC (DVd & PVd) in functional high-risk r/r MM across DREAMM-7 & DREAMM-8*

Vincent Rajkumar @VincentRK · Dec 9, 2025

Unprecedented. #ASH25

Tec-Dara in relapsed **myeloma**.

@mvmateos



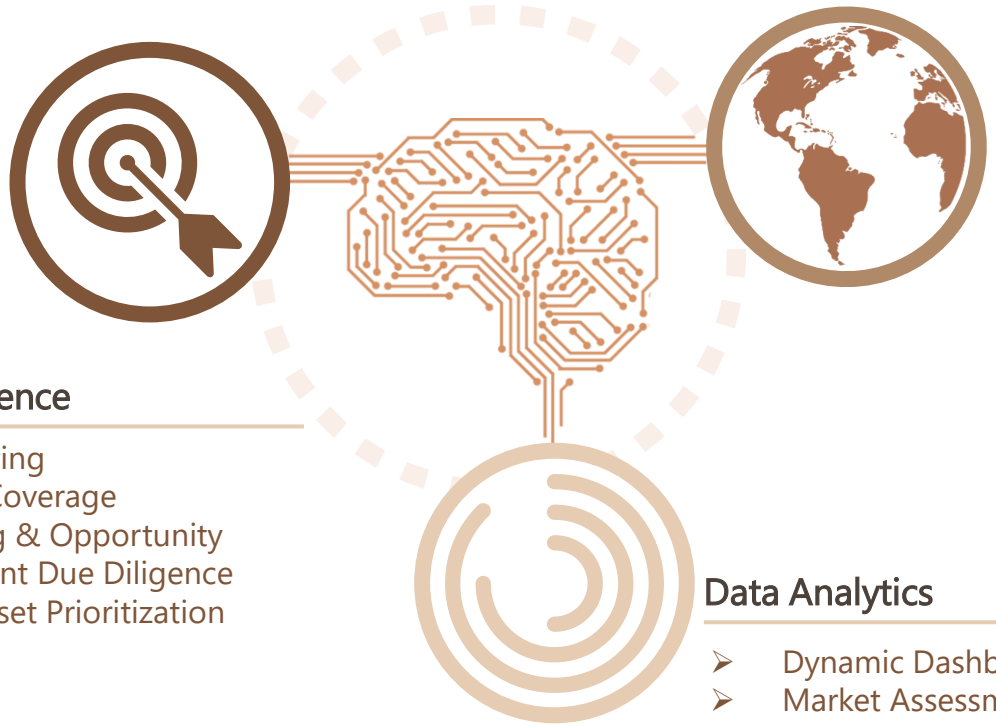
Luciano J Costa and 4 others

13 · 137 · 354 · 127K

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